

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in Tennessee

A licensed Personal Support Services Agency (PSSA) — private pay & TennCare CHOICES.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Tennessee model — read this first

In Tennessee, a business that provides non-medical personal care, homemaker, and companion help in people's homes is licensed as a Personal Support Services Agency (PSSA) — and here's the part that trips people up: it's licensed by the Tennessee Department of Mental Health and Substance Abuse Services (TDMHSAS), NOT the health department, under TCA Title 33 and rule 0940-05-38. (Skilled/medical home health is a separate Home Care Organization license from the Health Facilities Commission.) The PSSA license is refreshingly light: no on-site survey — just an electronic “desk audit.” This guide walks the whole path, broken down so a first-timer can follow it.

Pick the right door — PSSA, not the health department. A purely non-medical agency in Tennessee is a Personal Support Services Agency licensed by TDMHSAS (Title 33, rule 0940-05-38) — you apply to a TDMHSAS regional Office of Licensure. If your clients are 50%+ intellectual/developmental disabilities, you'd instead go through DIDD; a senior-focused non-medical agency goes through TDMHSAS. Only if you add skilled/medical care (nursing under physician orders) do you need a Home Care Organization license from the Health Facilities Commission — a separate, heavier, surveyed license.

Good news: no on-site survey. A PSSA doesn't require a life-safety/fire inspection, and the initial review is an electronic desk audit rather than an on-site survey. That makes Tennessee one of the faster non-medical home-care licenses to obtain.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with TDMHSAS and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed non-medical (PSSA) agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance & bonding (Part 3)
3	Get the PSSA license	TDMHSAS application, the fee, the desk audit (Part 4)
4	Set up your operation	The designated responsible person + policies (Parts 4–5)
5	Recruit & screen workers	Training, background + registry checks (Part 5)
6	Set up EVV & operations	CareBridge EVV, policies, systems (Part 6)
7	Serve private-pay clients	Start earning while MCO contracting proceeds (Part 8)
8	Contract for TennCare (CHOICES)	Contract with the CHOICES MCOs (Part

		8)
9	Get your first clients	Discharge planners, the 9 AAADs, care managers (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Get your entity and policies in order, file the PSSA application with your regional TDMHSAS office, and pass the desk audit. Launch on private pay, then contract with the TennCare CHOICES managed-care plans for Medicaid volume.

PART 1

What This Business Is

In short: A licensed Personal Support Services Agency — workers who help clients with self-care and household tasks at home. Non-medical, private pay and TennCare CHOICES.

Tennessee's PSSA license (rule 0940-05-38) covers personal support services delivered in a client's home to a person with substantial limitations in two or more major life activities who does not need nursing-level acute care. Covered tasks: self-care (eating, dressing, toileting, bathing, mobility, transfers) and household tasks (housekeeping, laundry, meal prep, shopping, bill-paying, community access). Housekeeping-only, transportation-only, or delivery-only businesses (with no other assistance) are exempt and need no license.

PSSA (non-medical) vs. Home Care Organization (skilled)

	PSSA (this guide)	Home Care Org - Home Health (skilled)
What it is	Personal support: self-care + household tasks	Nursing/therapy under physician orders
Licensed by	TDMHSAS (Title 33)	Health Facilities Commission (Title 0720)
Survey	Desk audit — no on-site survey	On-site surveyor inspection
Medications	Assistance only, under RN general supervision	Nurses administer/manage medications

Key terms. PSSA = Personal Support Services Agency (your license). TDMHSAS = TN Dept. of Mental Health & Substance Abuse Services (your licenser). DIDD = Dept. of Intellectual & Developmental Disabilities (the alternate licenser if your clients are 50%+ I/DD). HFC = Health Facilities Commission (licenses skilled home health). TennCare = TN Medicaid. CHOICES = TennCare's long-term-services program. MCO = a TennCare managed-care plan. CareBridge = the CHOICES EVV system. DDA = Dept. of Disability & Aging (the aging network). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Tennessee's 65-and-over population is growing quickly across both its metro areas and its rural counties, and the state's lower cost base means a lean startup can reach break-even on a modest census — while the light-touch PSSA desk-audit license lets you open faster than in most states. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: TDMHSAS licenses you (desk audit), TennCare and the CHOICES MCOs pay you for Medicaid, background/registry checks screen your workers, and CareBridge verifies Medicaid visits.

TDMHSAS — Office of Licensure (your licensor)

The Tennessee Department of Mental Health and Substance Abuse Services licenses PSSAs under rule 0940-05-38, reviews your application by electronic desk audit, and oversees compliance. Regional Offices of Licensure: West TN (901) 543-7442; Middle TN (615) 532-6590; East TN (865) 594-6551; tn.gov/behavioral-health/licensing.

TennCare & the CHOICES MCOs (Medicaid)

TennCare delivers long-term in-home care through CHOICES, run by the managed-care organizations. You contract with each MCO you want to serve: BlueCare (BlueCross BlueShield of TN), UnitedHealthcare Community Plan, and Wellpoint (formerly Amerigroup). LTSS Help Desk: 1-877-224-0219.

Background & registry checks

Before a worker serves clients, you run a criminal background check (re-run every 2 years) and check the Tennessee Abuse Registry and the Sex Offender Registry (both annually and before any direct contact). No one listed on the Abuse Registry may serve.

EVV — CareBridge

For TennCare CHOICES personal care, the MCOs use CareBridge for Electronic Visit Verification (available to providers at no cost; third-party vendors allowed at your expense). A visit must be verified to be paid. Private-pay work isn't subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Tennessee you form the LLC by filing Articles of Organization with the Tennessee Secretary of State (sos.tn.gov/business-services). Tennessee prices the filing fee per member — \$50 per LLC member, with a \$300 minimum and a \$3,000 maximum — so a solo-owner LLC costs \$300. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization.** Online at sos.tn.gov/business-services. Choose a unique name and list a registered agent (a person or service with a physical Tennessee address who receives legal mail — this can be you).
- 2. Pay the filing fee.** \$50 per member, minimum \$300, maximum \$3,000 — so a single-member LLC is \$300. This per-member floor is higher than many states, so budget for it.
- 3. Adopt an Operating Agreement.** Your internal document setting out ownership, management, and how money moves — banks and payers will want to see it.
- 4. Calendar your annual report.** Tennessee LLCs file an annual report — \$50 per member, minimum \$300 — due by the first day of the fourth month after your fiscal-year close. Miss it and the state can administratively dissolve your LLC.

No newspaper publication in Tennessee. *Unlike New York, Tennessee has no LLC publication requirement — forming the entity is a straightforward online filing. Just remember the per-member fee floor: \$300 to form and \$300 a year to stay in good standing for a solo LLC.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with TennCare and bill the CHOICES managed-care plans. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Tennessee branches — First Horizon (Tennessee-based), Regions, Truist, or Pinnacle, or a local Tennessee credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends workers into clients' homes, so you need coverage built for that exact risk. Here is what to line up before you take your first client:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE itself (a worker's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by a worker — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home — families ask if caregivers are “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your workers driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Worker injuries (lifting and transferring cause most claims)	Required in TN at 5+ employees

Workers' comp kicks in at five. Tennessee generally requires workers' compensation once you have five or more employees — but many payers and clients expect it sooner, and one lifting injury without it can be ruinous. Ask your broker to quote it from your first hire.

Real places to get it

Buy through an independent agent or broker who writes home-care programs — carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local Tennessee agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Confirm the carrier writes non-medical home care in Tennessee before you rely on a quote.

What to ask the insurance agent

“Do you write non-medical PSSAs in Tennessee — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include non-owned & hired auto (my workers drive their own cars)?”

“Can you place an employee-dishonesty bond so I can tell clients my caregivers are bonded?”

“What coverage limits do the TennCare CHOICES MCOs, the VA, or long-term-care insurers require?”

“When do I need Tennessee workers' comp given my worker headcount, and can you set it up now?”

PART 4

Get Your PSSA License — Step by Step

In short: Assemble your application and policies, submit to your regional TDMHSAS Office of Licensure, pay the fee, and pass the electronic desk audit. No on-site survey.

4.1 Before you apply

- ☐ Entity formed; EIN and NPI obtained; insurance & bond bound
- ☐ A designated individual responsible for operations (Part 5)
- ☐ Your written policies & procedures — client care, worker training, background/registry checks, complaints, incidents, emergency
- ☐ An attestation about your client base (to confirm TDMHSAS is the right licenser)

4.2 The application

- 1. Confirm the department.** File a written attestation about your client mix — a senior-focused non-medical agency licenses through TDMHSAS (rule 0940-05-38-.04).
- 2. Submit the application package.** Initial Application, Fact Sheet, Background Check & Privacy Statement form, and a Financial Statement — to your regional TDMHSAS Office of Licensure.
- 3. Pay the fee.** The license fee is set by rule 0940-05-02-.05 by number of service categories; a standalone PSSA is typically one category — about \$810 (confirm your category count/fee on the invoice).
- 4. Pass the desk audit.** The initial review is an electronic desk audit — no on-site survey. The license is valid up to one year and renews annually.

4.3 Timeline

TDMHSAS doesn't publish a fixed turnaround; the desk-audit model tends to be faster than a surveyed license. Submit a complete package and start your MCO contracting and worker hiring in parallel. Launch on private pay as soon as you're licensed.

PART 5

Your Responsible Person & Workers — by Position

In short: *Tennessee's PSSA rules are light on credentials but specific on screening. Here they are, one role at a time.*

5.1 Your responsible person / administrator

The licensee must designate an individual responsible for the operation, and operate under written policies and procedures. Tennessee does NOT prescribe a specific degree or credential for this role — a genuinely light-touch standard compared to home-health administrator rules. This can be you.

5.2 Your personal support workers — training & competency (0940-05-38-.08)

- **Age:** all workers must be at least 18.
- **Pre-service competency:** training in about seven areas — including observing, reporting, and documenting changes, and detecting/reporting/preventing abuse, neglect, and exploitation — plus service-specific training in self-care, household management, and community living.
- **Annual training:** at least annual job-related training. There's no fixed state hour count (unlike the home-health-aide standard), so build a solid orientation and skills check-off yourself.
- **Medication assistance:** only with documented training and under an RN's general supervision.

5.3 Background & registry checks (TCA § 33-2-1202)

1. **Run a criminal background check.** Within 10 days of employment and re-run every 2 years.
2. **Check the Abuse Registry — before any direct contact and annually.** No one listed may be hired or serve.
3. **Check the Sex Offender Registry — before direct contact and annually.**
4. **Confirm the fingerprint/vendor details.** TBI fingerprint checks and the vulnerable-persons registry are commonly used to satisfy the requirement — confirm the current scope with the Office of Licensure. Keep dated results in each file.

TB screening. *The PSSA rules don't explicitly mandate a TB test, but it's standard best practice and may be required by an MCO contract — confirm before you assume, and screen anyway as good practice.*

5.4 Building your Tennessee screening-and-training pipeline (the practical playbook)

Tennessee doesn't dictate a fixed training-hour count or a state certificate for personal support workers the way some states do — which is freeing, but it also means the burden is on YOU to build a screening-and-training pipeline that satisfies your policies, your MCO contracts, and any audit. Because there's no certified-aide bottleneck like a New Jersey CHHA course, your real constraint is speed of screening: the agency that clears a good worker fastest gets to say yes when a discharge planner calls. Here is how to keep the pipeline full and clean:

1. **Recruit continuously and verify age and identity first.** Every worker must be at least 18. Post on Indeed, local Facebook groups, and by word of mouth, offer a worker-referral bonus, and confirm identity and the right to work before anything else.
2. **Run the background and registry checks up front — and calendar the re-runs.** Criminal background check within 10 days of employment and again every 2 years; the Tennessee Abuse Registry and Sex Offender Registry before any direct client contact and annually. Anyone on the Abuse Registry cannot serve — full stop. TBI fingerprinting is commonly used; confirm the current scope with your Office of Licensure.
3. **Deliver your pre-service competency training and document it.** Cover the roughly seven required areas — including observing, reporting, and documenting changes, and detecting, reporting, and preventing abuse, neglect, and exploitation — plus service-specific training in self-care, household management, and community living. Keep a dated, signed skills check-off in each file.
4. **Schedule the annual refresher and keep records inspection-ready.** At least annual job-related training, plus documented medication-assistance training (under an RN's general supervision) for any worker who assists

with medications. Because Tennessee sets no fixed hour count, your written curriculum IS the standard the desk audit measures you against.

***Your training program is your compliance backbone.** In a state with no state-set hour count, a written orientation curriculum, a skills check-off signed before a worker works alone, and a dated annual-training log are exactly what the desk audit and any MCO audit look for. Build them once, use them every hire, and you turn a light-touch rule into a genuine quality advantage.*

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: Write your policies, set up CareBridge EVV for TennCare visits, and get your scheduling/billing systems in place. EVV is the gate to Medicaid payment.

6.1 Your policies & procedures

Build a P&P Manual to rule 0940-05-38: client rights, intake and the service plan, worker training and competency, background and registry checks, complaint handling, incident and abuse reporting, and emergency procedures. This is your operating backbone and what the desk audit reviews.

6.2 Set up EVV (before your first TennCare visit)

TennCare CHOICES uses CareBridge for Electronic Visit Verification. Onboard, set up your workers/clients/authorizations, capture every visit (GPS check-in/out with the six federal data points, ≥90% compliance expected), and bill against verified visits. Assign an EVV coordinator. Private-pay work isn't subject to EVV.

6.3 Systems

Choose a home-care platform for scheduling, records, invoicing, and payroll that works with CareBridge for your TennCare line.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via an MCO), you assess and set up the plan, a worker delivers the hours (with EVV for TennCare), you supervise, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a CHOICES care coordinator (Part 10). Take the basics — who, what tasks, how many hours, where, when, and who pays (private, CHOICES, VA, or LTC insurance).

Stage 2 — Assessment & the service plan

Do an in-home assessment and build a service plan: the tasks, schedule, and hours. For CHOICES clients, the MCO's care plan and authorization set the approved hours (personal care visits are capped at 2,580 hours/year in Groups 2/3); your plan aligns to it.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Match a screened, trained worker and start with a supervised introduction.

Stage 4 — Delivering the hours (with EVV for TennCare)

The worker delivers care; TennCare visits are captured in CareBridge. Keep visit notes and protect the client-worker match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

Supervise per your policy; monitor performance and changing needs; run your quality program; and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, update the service plan; for CHOICES clients, coordinate reassessments and new authorizations with the MCO care coordinator. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any MCO, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, TennCare (CHOICES), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid by contracting with the TennCare CHOICES managed-care plans.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately; ~\$31/hr in TN (2024, below national)
Medicaid — TennCare CHOICES	Long-term services through the MCOs	Personal care/attendant/homemaker; requires MCO contracts & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 3, Optum)

8.1 Private pay — your launch market

The day your license issues, serve private-pay clients — families paying out of pocket, typically billed hourly with a visit minimum. Tennessee is a lower-cost market: non-medical care runs about \$31/hour (2024 cost-of-care data — verify the current figure). Set your rate above your fully-loaded worker cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met — you bill the family or the insurer.

8.2 Medicaid — TennCare CHOICES

TennCare delivers long-term in-home care through CHOICES (for adults 65+ and adults 21+ with physical disabilities), run by the managed-care plans. CHOICES covers personal care visits, attendant care, homemaker, in-home respite, and companion care. You earn Medicaid revenue by contracting with the CHOICES MCOs.

1. **Hold your PSSA license first (Part 4).**
2. **Get an NPI and TennCare/MCO provider enrollment.**
3. **Sign a Participating Provider Agreement with each MCO.** BlueCare, UnitedHealthcare Community Plan, and Wellpoint — complete credentialing and ownership/disclosure per 42 CFR Part 455 (Wellpoint routes enrollment via Availity).
4. **Set up CareBridge EVV (Part 6)** before you deliver a CHOICES visit. Members may also self-direct via Consumer Direct Care Network Tennessee — know it exists.

CHOICES personal-care and attendant-care rates are set in TennCare's schedule and each MCO contract — request the current rate table from each MCO and TennCare; don't rely on a quoted number.

8.3 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. Tennessee is in Region 3, administered by Optum (888-901-6613).
- **Aid & Attendance:** an increased VA pension the veteran can apply toward your private-pay care (verify current amounts on VA.gov).

PART 9

What It Costs & How Long It Takes

In short: Budget the license, insurance & bonding, worker training, EVV, payroll float, and (optionally) accreditation. The PSSA desk-audit model keeps startup simple.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
PSSA license	Initial PSSA license (1 service category)	~\$810 (confirm)
Business setup	SOS LLC min \$300 + \$300/yr report, EIN free, NPI free, local business tax, legal	~\$300 + local + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Worker screening/training	Background + registry + your training program	Per worker
EVV & software	CareBridge (free) + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay workers before TennCare/insurers pay you	Working capital
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay workers weekly, but TennCare and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 Timeline

Weeks	Entity & policies	Form the LLC, write your P&P to 0940-05-38
Weeks	Insurance	Bind coverage & bond
Submit	PSSA application	Application + fee to your regional TDMHSAS office; pass the desk audit
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Parallel	Contract TennCare	MCO Participating Provider Agreements + CareBridge EVV
Ongoing	Referrals & census	Discharge planners, AAADs, CHOICES care coordinators

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$31/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that catch underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay workers every week or two, but the TennCare CHOICES MCOs and long-term-care insurers pay you weeks after you bill. On a growing Medicaid caseload you can be out tens of thousands of dollars in wages you've already paid. Keep 8–12 weeks of payroll in reserve, or lean on private pay (which pays up front) while your Medicaid line ramps.
- **Unbillable overhead:** your own time, a scheduler/coordinator, office and phone, background and registry checks, and any RN general-supervision time your policies or MCO contracts require — none of it is billable, all of it is real.
- **Turnover churn:** every worker who quits costs you re-recruiting, re-screening, and re-training — and often the client they served. Budget for it, and spend on retention instead; it's cheaper than replacement.
- **Denied and delayed claims:** a CHOICES claim that doesn't match a verified CareBridge EVV visit, or misses timely filing, simply doesn't get paid. Reconcile every remittance and chase EVV exceptions weekly — unreconciled claims are money you earned and lost.
- **The per-member annual report:** Tennessee's \$300-minimum annual report is easy to forget — miss it and the state can administratively dissolve your LLC, which can jeopardize contracts and your license. Calendar it every year.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the 9 Area Agencies on Aging and Disability, CHOICES care coordinators, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-skilled hours their benefit doesn't cover; build reciprocal partnerships.
- **Area Agencies on Aging and Disability (AAADs):** Tennessee has 9 regional AAADs under the Department of Disability and Aging — statewide single-entry line 1-866-836-6678 — that field families needing in-home help and often administer Veteran-Directed Care. Get listed and known.
- **CHOICES care coordinators:** once contracted, the MCOs' care coordinators route member hours to in-network agencies.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-/independent-living communities refer supplemental one-on-one care.
- **TN 2-1-1:** list your services with TN 2-1-1 (dial 211) — a free statewide referral channel.

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the responsible person & workers

Who	Contact	What to verify / ask
TDMHSAS — Office of Licensure (PSSA)	West (901) 543-7442 · Middle (615) 532-6590 · East (865) 594-6551	The PSSA application; the fee/category count; the desk audit; your client-base attestation
Background & registry checks	TBI · TN Abuse Registry · Sex Offender Registry	The criminal check (every 2 yrs); the registry checks; the current vendor/scope
Health Facilities Commission (contrast)	(615) 741-7221 · tn.gov/hfc	Only if you add SKILLED care — the Home Care Organization license
City/County business tax	your jurisdiction	Your local business license/tax

Business, insurance & billing IDs

Who	Contact	What to verify / ask
TN Secretary of State	sos.tn.gov/business-services	Forming your LLC (\$50/member, min \$300); the annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppes.cms.hhs.gov	Your Type 2 NPI (for TennCare/insurance billing)
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.3	Liability + abuse + non-owned auto + dishonesty bond + workers' comp

Funding, EVV & referrals

Who	Contact	What to verify / ask
TennCare CHOICES / LTSS Help Desk	1-877-224-0219 · tn.gov/tenncare	Contracting; the current CHOICES rates; authorizations; timely filing
CHOICES MCOs	BlueCare 888-747-8955 · UnitedHealthcare 800-690-1606 · Wellpoint 833-731-2153	Participating Provider Agreements; credentialing; EVV
CareBridge (EVV)	via your MCO	EVV onboarding and compliance
Consumer Direct Care Network TN	consumerdirecttn.com	The member self-direction option
VA Community Care / Optum (Region 3)	888-901-6613 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging & Disability (9)	1-866-836-6678 · tn.gov/disability-and-aging	Local information & referral for older adults
TN Dept. of Disability and Aging (DDA)	(800) 535-9725	The state aging network / ADRC
TN 2-1-1	dial 211	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; MCO/VA requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to take TennCare (Medicaid)?

No. Many Tennessee agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — often the smarter way to start, because you earn immediately without the MCO contracting and CareBridge EVV that TennCare CHOICES requires. Add Medicaid later once you have cash flow and a team.

Is the PSSA license really licensed by the mental-health department, not health?

Yes — a purely non-medical, senior-focused agency is a Personal Support Services Agency licensed by TDMHSAS under Title 33 and rule 0940-05-38, reviewed by an electronic desk audit with no on-site survey. Only if you add skilled/medical care do you need a Home Care Organization license from the Health Facilities Commission.

You can do this

Opening a Tennessee non-medical home care agency is very doable — a light-touch PSSA license with a desk audit instead of an on-site survey, plus a real TennCare CHOICES market alongside private pay. Take it one rung at a time: form the business, write your policies to rule 0940-05-38, get the PSSA license from TDMHSAS, recruit and screen great workers, launch on private pay, contract with the CHOICES MCOs, and build referral relationships. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in TCA Title 33, rule 0940-05-38, and current TennCare information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with TDMHSAS, TennCare, and qualified counsel before you rely on it. Confirm the current PSSA fee/category, the CHOICES rates, and your EVV setup before you rely on them.